National Cancer Control Programme (NCCP) 00745 · clinician draft

# Dose-dense AC → weekly paclitaxel + 3-weekly trastuzumab

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

## AC · 14-day schedule

**Active medicines:** Doxorubicin + Cyclophosphamide

**Emetogenic risk:** High · AC

| Medicine | Dose / route | When |
|---|---|---|
| Aprepitant | 125 mg orally | Day 1 |
| Ondansetron | 16 mg orally | Day 1<br>Individual dose / QT / hepatic review |
| Dexamethasone | 12 mg orally | Day 1<br>No routine Days 2–4 dexamethasone in this NCCP AC antiemetic row |
| Olanzapine | 5 mg orally | Days 1–4<br>Review sedation, falls, interactions and glucose |
| Aprepitant | 80 mg orally daily | Days 2–3 |
| Metoclopramide | 10 mg orally up to three times daily | Subsequent days: duration and use to be confirmed<br>Caution with olanzapine: additive extrapyramidal effects; adjust / avoid for individual contraindications |

### Additional phase-specific support

- This AC antiemetic script is replaced at the taxane transition.
- Growth-factor support is required for dose-dense AC. Specify the product, dose and days under local policy.
- Select the taxane support plan at the phase transition; do not continue the AC antiemetic course by default.

**Before signing:** verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00745 v2a · p6 · NCCP antiemetics v8, Table 1 pp4–5

Sources checked 2026-10-25 · SACTCheck v0.84.18 · Regimen-level drafts; prescriber and oncology-pharmacy verification required